31001590

Clarityne®

5 mg/5 mL Syrup
Grape Flavour

BAYER

Brand of loratadine
Long-Acting, Non-Sedating Antihistamine
For Oral Use

PRODUCT INSERT

1. NAME OF THE MEDICINAL PRODUCT

Clarityne® Syrup Grape 5 mg/5 mL

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Each 5 ml of CLARITYNE® Syrup contains 5 mg of micronized loratadine

The quantity of sucralose in the loratadine syrup composition is 750 mg/ml. The amount of sucralose per 5 ml (5 mg) dose is 3.75 g.

3. PHARMACEUTICAL FORM

Clear, colourless to light yellow syrup.

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

CLARITYNE® Products are indicated for the relief of symptoms associated with allergic rhinitis, such as sneezing, nasal discharge (rhinorrhea) and itching, as well as ocular itching and burning. Nasal and ocular signs and symptoms are relieved rapidly after oral administration.

CLARITYNE® Products are also indicated for relief of symptoms and signs of chronic urticaria and other allergic dermatologic disorders.

4.2 Posology and method of administration

Adults and Children 12 years of age and over:

Two 5 ml spoonfuls (10 mg) once daily.

Children 2 to 12 years of age:

Body Weight >30 kg -- Two 5 ml spoonfuls (10 mg) syrup once daily.

Body Weight ≤30 kg -- One 5 ml spoonful (5 mg) syrup once daily.

Safety and efficacy of CLARITYNE® has not been established in children younger than 2 years of age.

Patients with severe liver impairment should be administered a lower initial dose because they may have reduced clearance of loratadine; an initial dose of 5 mg or 5 ml once daily, or 10 mg or 10 ml every other day is recommended.

4.3 Contraindications

CLARITYNE® Products are contraindicated in patients who are hypersensitivity to the active substance or to any of the excipients in these formulations.

4.4 Special warnings and precautions for use

CLARITYNE® Products should be administered with caution in patients with severe liver impairment (see section 4.2).

The administration of CLARITYNE® Products should be discontinued at least 48 hours before skin tests since antihistamines may prevent or reduce otherwise positive reactions to dermal reactivity index.

4.5 Interaction with other medicinal products and other forms of interaction

When administered concomitantly with alcohol, loratadine has no potentiating effects as measured by psychomotor performance studies.

Potential interaction may occur with all known inhibitors of CYP3A4 or CYP2D6 resulting in elevated levels of loratadine (see Section 5.2), which may cause an increase in adverse events.

4.6 Pregnancy and lactation

Safe use of CLARITYNE® Products during pregnancy has not been established; therefore, use only if the potential benefit justifies the potential risk to fetus. Since loratadine is excreted in breast milk, a decision should be made whether to discontinue nursing or discontinue the drug.

4.7 Effects on ability to drive and use machines

In clinical trials that assessed driving ability, no impairment occurred in patients receiving loratadine. However, patients should be informed that very rarely some people experience drowsiness, which may affect their ability to drive or use machines.

4.8 Undesirable Effects

In clinical trials in a paediatric population, children aged 2 through 12 years, common adverse reactions reported in excess of placebo were headache (2.7%), nervousness (2.3%), and fatigue (1%).

In clinical trials involving adults and adolescents in a range of indications including AR and CIU, at the recommended dose of 10 mg daily, adverse reactions with loratadine were reported in 2%of patients in excess of those treated with placebo. The most frequent adverse reactions reported in excess of placebo were somnolence (1.2%), headache (0.6%), increased appetite (0.5%) and insomnia (0.1%). Other adverse reactions reported very rarely during the post-marketing period are listed in the following table.

Immune system disorders	Anaphylaxis including angioedema
Nervous system disorders	Dizziness & convulsion
Cardiac disorders	Tachycardia, palpitation
Gastrointestinal disorders	Nausea, dry mouth, gastritis
Hepatobiliary disorders	Abnormal hepatic function
Skin and subcutaneous tissue disorders	Rash, alopecia
General disorders and administration site conditions	Fatigue

4.9 Overdose

Overdosage with loratadine increase the occurrence of anticholinergic symptoms. Somnolence, tachycardia and headache have been reported with overdoses.

In the event of overdose, general symptomatic and supportive measures are to be instituted and maintained for as long as necessary. Administration of activated charcoal as a slurry with water may be attempted. Gastric lavage may be considered. Loratadine is not removed by haemodialysis and it is not known if loratadine is removed by peritoneal dialysis. Medical monitoring of the patient is to be continued after emergency treatment.

5. PHARMACOLOGICAL PROPERTIES

5.1 Pharmacodynamic properties

Pharmacotherapeutic group: antihistamines – H1 antagonist, ATC code: R06A X13.

Loratadine, the active ingredient in CLARITYNE® products, is a tricyclic antihistamine with selective, peripheral H1-receptor activity.

Loratadine has no clinically significant sedative or anticholinergic properties in the majority of the population and when used at the recommended dosage.

During long-term treatment there were no clinically significant changes in vital signs, laboratory test values, physical examinations or electrocardiograms.

Loratadine has no significant H2-receptor activity. It does not inhibit norepinephrine uptake and has practically no influence on cardiovascular function or on intrinsic cardiac pacemaker activity.

5.2 Pharmacokinetic Properties

Absorption

Loratadine is rapidly and well-absorbed. Concomitant ingestion of food can delay slightly the absorption of loratadine but without influencing the clinical effect. The bioavailability parameters of loratadine and of the active metabolite are dose proportional.

Distribution

Loratadine is highly bound (97% to 99%) and its active metabolite moderately bound (73% to 76%) to plasma proteins. In healthy subjects, plasma distribution half-lives of loratadine and its active metabolite are approximately 1 and 2 hours, respectively.

Biotransformation

After oral administration, loratadine is rapidly and well absorbed and undergoes an extensive first pass metabolism, mainly by CYP3A4 and CYP2D6. The major metabolite-desloratadine (DL) is pharmacologically active and responsible for a large part of the clinical effect. Loratadine and DL achieve maximum plasma concentrations (Tmax) between 1–1.5 hours and 1.5–3.7 hours after administration, respectively.

Elimination

Approximately 40% of the dose is excreted in the urine and 42% in the faeces over a 10 day period and mainly in the form of conjugated metabolites. Approximately 27% of the dose is eliminated in the urine during the first 24 hours. Less than 1% of the active substance is excreted unchanged in active form, as loratadine or DL. The mean elimination half-lives in healthy adult subjects were 8.4 hours (range = 3 to 20 hours) for loratadine and 28 hours (range = 8.8 to 92 hours) for the major active metabolite.

Renal Impairment

In patients with chronic renal impairment, both the AUC and peak plasma levels (Cmax) increased for loratadine and its active metabolite as compared to the AUCs and peak plasma levels (Cmax) of patients with normal renal function. The mean elimination half-lives of loratadine and its active metabolite were not significantly different from that observed in normal subjects. Haemodialysis does not have an effect on the pharmacokinetics of loratadine or its active metabolite in subjects with chronic renal impairment.

Hepatic impairment

In patients with chronic alcoholic liver disease, the AUC and peak plasma levels (Cmax) of loratadine were double while the pharmacokinetic profile of the active metabolite was not significantly changed from that in patients with normal liver function. The elimination half-lives for loratadine and its active metabolite were 24 hours and 37 hours, respectively, and increased with increasing severity of liver disease.

Elderly

The pharmacokinetic profile of loratadine and its active metabolite is comparable in healthy adult volunteers and in healthy geriatric volunteers.

5.3 Preclinical safety data

Preclinical data reveal no special hazard based on conventional studies of safety, pharmacology, repeated dose toxicity, genotoxicity and carcinogenic potential.

In reproductive toxicity studies, no teratogenic effects were observed. However, prolonged parturition and reduced viability of offspring were observed in rats at plasma levels (AUC) 10 times higher than those achieved with clinical doses.

No evidence of mucous membrane irritation was observed after daily administration of up to 12 tablets (120 mg) of oral lyophilisates into the hamster cheek pouch for five days.

6. PHARMACEUTICAL PARTICULARS

6.1 List of Excipients

Edetate disodium, flavor, glycerol, maltitol, sodium dihydrogen phosphate dihydrate, phosphoric acid, propylene glycol, purified water, sorbitol, sucralose, sodium benzoate (0.05%w/v) as preservative.

6.2 Incompatibilities

No incompatibilities.

6.3 Special Precautions for storage

Store below 30°C. Syrup bottle should be protected from excessive moisture. Keep out of reach of children. Controlled Medicine.

6.4 Nature and Contents of container

In bottle of 60 ml.

7. PRODUCT REGISTRATION HOLDER

Bayer Co. (Malaysia) Sdn Bhd
25-03 & 25-04, Level 25, IMAZIUM, No. 8, Jalan SS21/37, Damansara Uptown, 47400 Petaling Jaya, Selangor

8. MANUFACTURER

Delpharm Montreal Inc.
3535 Route Trans Canada Highway, Pointe-Claire, Quebec, Canada, H9R 1B4

9. REGISTRATION NO.

MAL10080045ACSZ

10. DATE OF REVISION OF TEXT

March 2024

Bayer